

Targeted therapies for ovarian cancer

Targeted therapies interfere with the way tumour cells grow and divide. Find out about the drugs used to treat ovarian cancer, fallopian tube cancer and primary peritoneal cancer.

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What are targeted therapies?

Targeted therapy uses drugs to find and attack cancer cells. There are different types of targeted therapy drug. Each type targets something in or around the cancer cell that is helping it grow and survive.

About targeted therapy for cancer of the ovary, fallopian tube or peritoneum

Your cancer doctor and specialist nurse can explain whether targeted therapy is suitable for you. They can explain how long you will have targeted therapy for. You may have it for as long as it is keeping the cancer away or controlling it.

We have more general information about [targeted therapies](#).

Bevacizumab (Avastin®)

You may have [bevacizumab](#) if either:

- the cancer has spread
- the cancer has come back.

You will have bevacizumab with [chemotherapy](#). You usually have it every 3 weeks. You have it into a vein as a drip (infusion).

You continue having bevacizumab after chemotherapy ends. You may have it for 1 year or longer if it is working for you and any side effects can be managed. This is called maintenance treatment. Sometimes you have bevacizumab with another targeted therapy drug called [olaparib](#).

PARP inhibitor drugs

These drugs block a protein called PARP. The PARP protein helps damaged cells to repair themselves. Without the PARP protein, the cancer cells may become too damaged to survive.

Some cancer cells have a genetic change called homologous recombination deficiency (HRD). They are called HRD positive cells. PARP inhibitors are even more effective against HRD positive cells.

You start taking PARP inhibitors after chemotherapy. You usually start them a few weeks after your last chemotherapy treatment. You take PARP inhibitors as tablets or capsules every day.

PARP inhibitors include:

- [olaparib](#) (Lynparza[®])
- [niraparib](#) (Zejula[®])
- [rucaparib](#) (Rubraca[®]).

You may have these drugs as maintenance treatment if the cancer responded to chemotherapy. You may have a PARP inhibitor after your first course of chemotherapy. Or you may have it after chemotherapy if the cancer comes back if and you did not have a PARP inhibitor before.

You may have olaparib, with or without bevacizumab, if:

- you have changes in the [BRCA1 or BRCA2 genes](#)
- tests show the cancer cells are HRD positive.

Side effects of targeted therapy drugs

Your cancer doctor, specialist nurse or pharmacist will explain the side effects of the drug you are having.

They will explain how some side effects can be controlled or managed. Always tell them about your side effects, especially if they do not improve or get worse. It is important to follow any advice your doctor, nurse or pharmacist gives you.

Some general side effects of targeted therapy drugs are:

- a low number of blood cells – this can increase the risk of [infection](#), anaemia and bleeding or bruising
- [tiredness](#)
-

[feeling sick](#) (nausea)

- loss of appetite and changes to your taste
- high blood pressure

Rarely, [bevacizumab](#) can cause an allergic reaction. This is more likely with the first or second infusions. Your nurse will give these infusions more slowly so they can monitor you closely.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our ovarian cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

Ovarian cancer: recognition and initial management. Clinical guideline [CG122] Published: 27 April 2011
Last updated: 02 October 2023 www.nice.org.uk/guidance/cg122 (accessed May 2024)

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Date reviewed

Reviewed: 01 March 2025 | Next review: 01 March 2029

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help